



DIAGNOSTIC IMAGING NETWORK

4910 Van Nuys Blvd. STE 108, Sherman Oaks, CA 91403 ♦ Tel: 818-986-8215
8641 Wilshire Blvd. STE 105, Beverly Hills, CA 90211 ♦ Tel: 310-919-3330
800 S. Central Ave, STE 100B, Glendale, CA 91204 ♦ Tel: 818-548-8333
13311 Victory Blvd. Van Nuys, CA 91401 ♦ Tel: 747-777-8494
403 S. Glendale Ave, Glendale, CA 91205 ♦ Tel: 818-546-1929

Patient Name : JUAREZ, JUAN
Date of Birth : 07/25/1978
Referring Physician : TSAI, CHRIS
Study Description : MRI C/SPINE

Patient ID : 254698
Study Date : 08/30/2024 17:41:43

CLINICAL HISTORY: Neck pain.

TECHNIQUE: Unenhanced MRI of the cervical spine was imaged in sagittal projection. Axial and coronal images from C1 through T1 were also obtained. The sequence was obtained by established institution protocol.

FINDINGS: There is loss of normal lordotic curvature cervical spine. The bone marrow signal is within normal limits. The height of the vertebral bodies is preserved. There is no paravertebral soft tissue abnormality.

C1-2: The atlantoaxial alignment is normal. There is no acute fracture or subluxation. There is no central canal stenosis.

C2-3: The disc height and signal is within normal limits. There is no disc herniation, central canal stenosis or neural foraminal narrowing. There is no significant facet disease.

C3-4: The disc height and signal is within normal limits. A 2.5 mm broad based posterior disc herniation resulting in mild central canal stenosis. There is impingement of the ventral aspect of the thecal sac at this level. There is no significant neural foraminal stenosis. There is no significant facet disease.

C4-5: The disc height and signal is within normal limits. A 2.0 mm broad based posterior disc herniation resulting in mild central canal stenosis. There is impingement of the ventral aspect of the thecal sac at this level. There is extension of the disc into left neural foramina resulting in left neural foraminal stenosis. There is no significant facet disease.

C5-6: The disc height and signal is within normal limits. A 3.0 mm broad based posterior disc herniation resulting in moderate central canal stenosis. There is impingement of the ventral aspect of the thecal sac at this level. There is extension of the disc into the left neural foramina resulting in left neural foraminal stenosis. There is no significant facet disease.

C6-7: The disc height and signal is within normal limits. A 2.5 mm broad based posterior disc herniation resulting in mild central canal stenosis. There is impingement of the ventral aspect of the thecal sac at this level. There is extension of the disc into left neural foramina resulting in left neural foraminal stenosis. There is no significant facet disease.

C7-T1: The disc height and signal is within normal limits. There is no disc herniation, central canal stenosis or neural foraminal narrowing. There is no significant facet disease.

IMPRESSION:

1. C3-4: The disc height and signal is within normal limits. A 2.5 mm broad based posterior disc herniation resulting in mild central canal stenosis. There is impingement of the ventral aspect of the thecal sac at this level. There is no significant neural foraminal stenosis. There is no significant facet disease.
2. C4-5: The disc height and signal is within normal limits. A 2.0 mm broad based posterior disc herniation resulting in mild central canal stenosis. There is impingement of the ventral aspect of the thecal sac at this level. There is extension of the disc into left neural foramina resulting in left neural foraminal stenosis. There is no significant facet disease.
3. C5-6: The disc height and signal is within normal limits. A 3.0 mm broad based posterior disc herniation resulting in moderate central canal stenosis. There is impingement of the ventral aspect of the thecal sac at this level. There is extension of

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the disc into the left neural foramina resulting in left neural foraminal stenosis. There is no significant facet disease.

4. C6-7: The disc height and signal is within normal limits. A 2.5 mm broad based posterior disc herniation resulting in mild central canal stenosis. There is impingement of the ventral aspect of the thecal sac at this level. There is extension of the disc into left neural foramina resulting in left neural foraminal stenosis. There is no significant facet disease.

5. There is loss of normal lordotic curvature cervical spine. This is a nonspecific finding which may be positional, due to spasm or ligamentous injury. Clinical correlation is recommended.

Thank you for your referral.

Electronically signed by: KAMRAN KOOCHEK, M.D., DABR, M.S.

Radiologist

T: 09/01/24 #1 KK/am